

# Day 3 — Tridosha Theory in Depth + Modern Engagement

**Module:** Ayurveda (Senior) · **Time:** 45 min

## Learning objective

Students articulate the Tridosha framework at classical depth + engage with the modern scholarly literature on its biomedical validity.

## Materials

- Tridosha reference table
- Printed extract: brief abstract of a study attempting biomarker validation of dosha types (e.g. Bhalerao et al. or similar)

## Flow

### Warm-up (5 min)

Recall: 3 doshas with their elemental basis (built from Module 2 Panchabhuta).

### Core (25 min)

#### 1. Classical Tridosha (depth):

| Dosha | Elements (mahābhūta) | Five sub-types                                   | Primary site      | Function                           |
|-------|----------------------|--------------------------------------------------|-------------------|------------------------------------|
| Vāta  | ākāśa + vāyu         | Prāṇa, Udāna, Samāna, Vyāna, Apāna               | Colon (pakvāśaya) | Movement (nerve impulses, breath)  |
| Pitta | tejas + ap           | Pācaka, Rañjaka, Sādhaka, Ālocaka, Bhrājaka      | Small intestine   | Transformation (digestion, vision) |
| Kapha | ap + pṛthvī          | Avalambaka, Kledaka, Bodhaka, Tarpaka, Ślaiṣmaka | Stomach (āmāśaya) | Structure, lubrication, immunity   |

**Note:** each dosha has FIVE sub-types with specific anatomical locations. The framework is far more granular than “Vāta = wind.”

#### 2. Prakṛti vs Vikṛti:

- *Prakṛti* = constitution (set at birth).
- *Vikṛti* = current state (can be out of balance).
- Ayurveda diagnoses vikṛti, prescribes return-to-prakṛti.

### 3. The modern scholarly engagement:

- **Prasher et al. (2008).** “Genome-wide expression analysis of Prakriti types.” Compared gene expression in self-identified Vāta / Pitta / Kapha types. Found *some* differences. Replication mixed.
  - **Patwardhan (2014).** *Bridging Ayurveda with evidence-based scientific approaches.* Argues for “Ayurgenomics” — combining Ayurvedic typology with genomic and metabolomic data. Promising; not conclusive.
  - **Critical view:** even the modest published correlations may be confounded by self-identification bias. Dosha typology may correlate with personality traits more than with biomedical biomarkers.
4. **The honest framing:** the Tridosha framework is *useful* clinically for many practitioners. The *biomedical-classification* claim has mixed evidence. These are different claims. Don’t conflate.

### Discussion (10 min)

- “Is Tridosha ‘true’ in the same way the periodic table is true? In what way might it be a different kind of framework?”
- Push toward: clinical typologies (DSM personality types, Sheldon’s somatotypes) often have modest biomarker correlates but real clinical utility. Tridosha may be in this category.

### Wrap-up (5 min)

- Preview Day 4: “Tomorrow — the broader modern evidence base for Ayurveda.”

### Differentiation

- **Tier up:** Read the Prasher et al. (2008) abstract carefully. Critique the methodology.
- **Tier down:** Focus on classical Tridosha + the framing distinction (clinical vs biomedical).

### Teacher notes

- The Tridosha “is it true?” question doesn’t have a clean yes/no. Stay with the framing distinction.
- Some students will want Tridosha to be either ALL true or ALL false. Both positions are too crude.

### Citation(s) used

- Caraka Sūtra-sthāna 18 on dosha sub-types.
- Patwardhan (2014). *EPMA Journal* 5(1), 19.
- Prasher et al. (2008) on Prakriti genomics (debated).